BILIARY, SPLEEN & PANCREAS IMAGING — Revision Table

Bold = high-yield. CT mainstay; US best for gallstones; MRCP replaced diagnostic ERCP/PTC.

BILIARY SYSTEM

Topic	Key points
Modalities	**US = primary/best all-purpose**; HIDA (biliary atresia, cystic duct obstruction); **MRCP replaced diagnostic ERCP/PTC**
US normals	Stones >1–2mm seen; **normal CBD ≤7mm, wall <3mm**; intrahepatic ducts not seen; fasting needed
HIDA	GB+CBD+duodenum in 1h. **Cystic duct obstruction = GB NOT seen, CBD+duodenum seen**
ERCP	Mainly therapeutic; **complications: pancreatitis, perforation, bleeding, sepsis**
PTC	Better with dilated ducts; **complications: bleeding, septicaemia, biliary peritonitis**
Gallstones	10–20% radio-opaque, **60–80% CT; US = echogenic + acoustic shadowing**; US unreliable for duct stones
GB polyp	Echogenic, **NO shadowing**; **>10mm → cholecystectomy**
Acute cholecystitis (US)	**Wall >3mm, distended >40mm, sludge/stone, wall oedema, +Murphy's**; wall oedema = acute vs chronic
Obstructive jaundice	Biliary dilatation (may lag 48h); **US more sensitive than CT**; commonest = CBD stone, CA head pancreas

SPLEEN

Topic	Key points
Imaging	US iso-echoic to liver; CT/MRI
Masses	Cyst (**hydatid**), abscess, **tumour = lymphoma > metastases**
Splenomegaly	**>13cm**; lymphoma, portal HTN, chronic infection, haemolytic anaemia, leukaemia
Trauma	**Most injured organ in blunt trauma**; **CT standard** (+ bleeding, liver/left kidney)

PANCREAS

Topic	Key points
Imaging	**CT main (not affected by air)**; US limited; **normal duct 2mm**; feathery outline (CT)
Masses	Carcinoma, LN, abscess/pseudocyst, focal pancreatitis
Adenocarcinoma	Commonest; **2/3 in head (early dx — duct dilatation); tail = late/advanced**; CT: low-density, fat obliteration, **vascular encasement = unresectable**
Endocrine tumour	Small, don't deform outline; **hypervascular → angiography/dynamic CT/MRI**
Acute pancreatitis	Diffusely enlarged + low density (oedema/necrosis); **CT to stage (CTSI), exclude abscess/pseudocyst/carcinoma**
Pseudocyst	Fluid cystic; **many resolve 4–6 weeks**; US/CT follow-up
Chronic pancreatitis	**Duct dilatation + beading, calcifications, atrophy, pseudocysts**